

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°143 /MVE-BDBV/04/10/2026

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**HIGHLIGHTS** (last 24 hours)

In the last 24 hours, 59 new confirmed cases of Ebola Virus Disease (EVD) have been reported, including 23 deaths. Community transmission of COVID-19 cases was reported. These cases originated from the provinces of North Kivu (33 cases), Ituri (23 cases), Haut-Uélé (2 cases), and Tshopo (1 case). In addition, 11 deaths were recorded in Ebola Treatment Centers (ETCs).

A total of 331 new samples were analyzed; 59 of these samples tested positive for the virus Bundibugyo, or 17.8% positivity.

ACCUMULATIVE CASE 8,603	ACCUMULATIVE DEATH CONFIRMED 4,148	LETHALITY 48.2%	PATIENTS IN ISOLATION/CTE 896	ACCUMULATIVE HEALED 2,235	TRACKING RATES CONTACTS (Of the Day) 79.7%
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Affected provinces and affected health zones

**7 Provinces affected**

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



64 out of 167
health zones affected
(38.3%)

Summary of key points from the last 24 hours

At the end of the day on October 4, 2026, 59 new confirmed cases of Ebola Virus Disease (EVD) including 23 community deaths were reported in 4 provinces: North Kivu (33 cases), Ituri (23 cases), Haut-Uélé (2 cases) and Tshopo (1 case).

In addition, 11 deaths were recorded in the CTEs (4 in Ituri, 3 in North Kivu, 3 in Haut-Uélé) and in Tshopo (1), bringing the total number of confirmed deaths for the period considered to 34.

Nevertheless, 5 patients (4 in Ituri and 1 in Haut-Uélé) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

As of October 4, 2026, the DRC has recorded 8,603 confirmed cases and 4,148 deaths (case fatality rate: 48.2 %). Ituri remains the epicenter of the epidemic with 75.0% of the cumulative confirmed cases reported and 39.0% of new confirmed cases. On the other hand, the province of North Kivu has a case fatality rate higher than the national average, at 58.1%.

64 out of 167 health zones (38.3%), spread across seven provinces, have been affected since the start of the epidemic.

Table 1. Distribution of confirmed cases and deaths by affected province, as of October 4, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	23	6450	2,968	46.0%	28/36 (77.8%)
North Kivu	33	1729	1005	58.1%	17/34 (50.0%)
Haut-Uélé	2	359	152	42.3%	7/13 (53.8%)
Tshopo	1	50	17	34.0%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	10	4	40.0%	3/11 (27.3%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
Total	59	8,603	4,148	48.2%	64/167 (38.3%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 4, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6450	2968	46.0%	23	12	4	16
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	208	46	22.1%	4	0	0	0
Boga	2	2	100.0%				0
Bunia	1779	584	32.8%	10	6	1	7
Damascus	40	9	22.5%				0
Drodro	15	7	46.7%				0
Fataki	83	31	37.3%				0
Gethy	9	5	55.6%	2	2	0	2
Kambala	2	0	0.0%				0
Kilo	38	12	31.6%				0
Komanda	175	107	61.1%				0
Lita	294	147	50.0%	3	1	0	1
Logo	13	5	38.5%				0
Lolwa	34	9	26.5%				0
Mahagi	4	1	25.0%				0
Mambasa	37	16	43.2%	1	1	0	1
Mandima	176	84	47.7%				0
Mangala	365	157	43.0%	0	0	1	1
Mongbwalu	707	335	47.4%			1	1
Nia-Nia	311	175	56.3%	1	1	0	1

Nizi	786	282	35.9%					0
Nyankunde	130	37	28.5%					0
Rimba	13	5	38.5%					0
Rwampara	1114	380	34.1%	2	1	1		2
Tchomia	76	25	32.9%					0
To ventilate	N / A	493	N / A					0
North Kivu	1729	1005	58.1%	33	10	3		13
Blessed	393	260	66.2%	8	3	1		4
Good	20	7	35.0%					0
Butembo	280	180	64.3%	5	2	2		4
Goma	1	0	0.0%					0
Kalunguta	60	21	35.0%					0
Katwa	600	370	61.7%	6	3			3
Kayna	3	2	66.7%					0
Kyondo	36	20	55.6%					0
Lubero	22	11	50.0%					0
Mabalako	46	18	39.1%	1	1			1
Alimbongo	4	2	50.0%					0
Manguredjipa	13	5	38.5%					0
Masereka	32	11	34.4%	1				0
Musienene	138	69	50.0%	10	1			1
Mutwanga	16	3	18.8%					0
Oicha	31	16	51.6%					0
Vuhovi	34	10	29.4%	2				0
Upper Uele	359	152	42.3%	2	1	3		4
Boma	43	18	41.9%					0
Mangbetu	1	1	100.0%					0
Dungu	3	1	33.3%					0
Gombari	102	44	43.1%					0
Pawa	90	43	47.8%	0	0	3		3
Rungu	4	2	50.0%					0
Wamba	116	43	37.1%	2	1	0		1
Tshopo	50	17	34.0%	1	0	1		1
Bafwasende	7	1	14.3%					0
Kabondo	5	1	20.0%					0
Lubunga	1	0	0.0%					0
Makiso - Kisangani	27	11	40.7%			1		1
Mangobo	8	3	37.5%	1	0	0		0
Tshopo	1	0	0.0%					0
Wanie - Rukula	1	1	100.0%					0
South Kivu	3	1	33.3%	0	0	0		0
Miti-Murhesa	3	1	33.3%					0
Bas Uélé	10	4	40.0%	0	0	0		0

Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Ganga	3	2	66.7%				0
South Ubangi	2	1	50.0%	0	0	0	0
Bulu	2	1	50.0%				0
Total	8603	4148	48.2%	59	23	11	34

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for October 4, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	16	3	19	16	1	2	0	17	13
Bas Uélé	91	2	93	5	2	86	0	7	0
Ituri	1038	69	1107	67	69	971	0	136	55
North Kivu	657	30	687	61	30	596	0	91	44
South Ubangi	36	0	36	0	0	36	0	0	
Tshopo	49	0	49	5	0	44	0	5	0
Total General	1887	104	1991	154	102	1,735	0	256	112

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

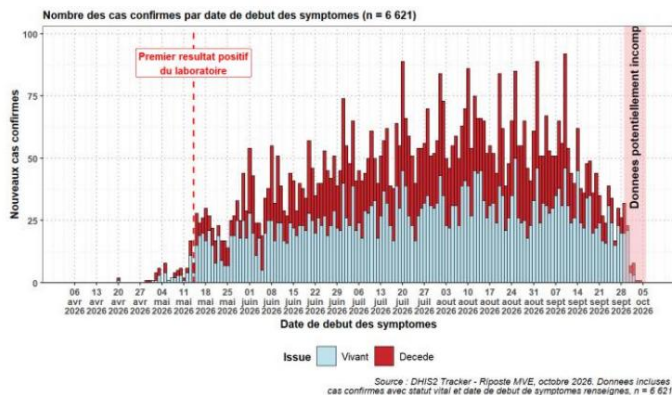
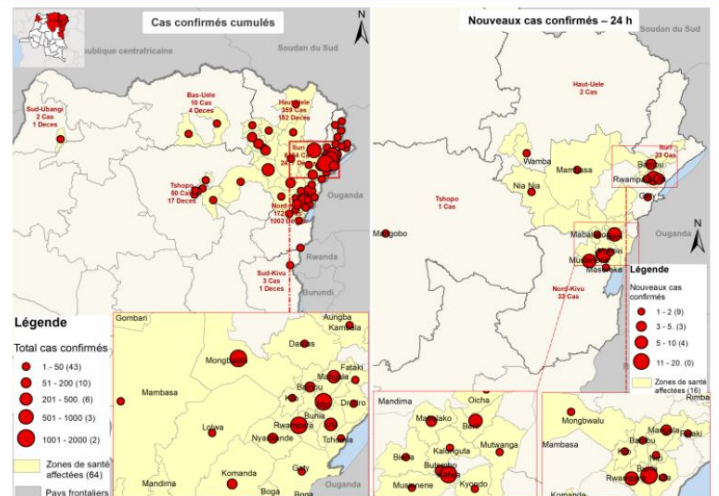


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

By the end of October 4th, **1991** alerts (100%) had been verified, of which **256** (12.9%) were validated as suspected cases. Of these, **256** (100%) were investigated. Consequently, 112 suspected cases were transferred to the CT/CTE ([Table 3](#)).

Of the **30,973** contacts currently being monitored, **24,673** were seen in the last 24 hours, representing a follow-up rate of **79.7** %, up from the previous day (**73.8** %), but still below the required threshold of **95.0** %. This rate was **97.7% (43/44)** in Sud-Ubangi, **84.5% (120/142)** in Bas-Uélé, **83.4% (1,584/1,900)** in Haut-Uélé, **83.3% (665/798)** in Tshopo, **81.8% (11,701/14,299)** in Ituri, and **76.8% (10,560/13,790)** in Nord-Kivu.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Three (3) alerts were notified to the PoE/PoC during the day of October 4th, all in Ituri, one (1) of which was validated as a suspected case. The two alerts from the DGRPI Nizi PoC (Nizi Health Zone) were isolated and then invalidated after secondary screening, while the confirmed suspected case from the Mabakese PoC (Mambasa Health Zone) was investigated and transferred to the CT/CTE (investigation and transfer rate of 100.0%; 1/1).

Table 4. Status of key indicators for PoE/PoC by province, as of October 4, 2026

Indicators	Ituri	North Kivu	Tshopo	Overall
Report completeness	93.3% (56/60)	95.2% (20/21)	100.0% (18/18)	94.9% (94/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	132,791	84,226	17,840	234,857
% of travelers screened	95.9%	97.6%	99.5%	96.8%
% of travelers who washed their hands	97.2%	97.6%	97.8%	97.4%
% of travelers made aware	99.1%	97.6%	100.0%	98.6%
Number of alerts notified	3	0	0	3
Number of validated live alerts (suspected cases)	1	0	0	1
Number of bodies recovered today	0	0	0	0
% of suspected cases transferred to CTE/CT	100.0%	N / A	N / A	100.0%
% of lifeless bodies swabbed/secured	N / A	N / A	N / A	N / A
% of suspected cases investigated within 4 hours	100.0%	N / A	N / A	100.0%

2.2.2. Challenges

In North Kivu, the Butuhe Point of Control (PoC) has been out of service since the security incident of September 26 (95.2% completeness; 20/21), 2.4% of travelers refused screening, and PoC providers along the Butembo axis remain exposed to a persistent security threat; alerts from the PoCs are not being addressed promptly by the alert units. In Tshopo, 0.5% of travelers refused screening and 2.2% refused handwashing.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 23 new positive results** (11 living and 12 dead) out of 146 samples received and analyzed (**positivity: 15.8%**).
- **North Kivu: 33 new positive results** (23 living and 10 dead) out of 161 samples received and analyzed (**positivity: 20.5%**).

- **Haut-Uélé: 2 new positive results** (1 living and 1 dead) out of 15 samples received and analyzed (positivity: 13.3%).
- **Tshopo: 1 new positive result received** (1 live sample) out of 4 samples received and analyzed (positivity: 25.0%).
- **Bas Uélé: 5 samples** (2 blood and 3 deaths) received and analyzed at the Buta laboratory, **all returned negative.**

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 20 out of 32 rings were opened (62.5%), 88 out of 88 households were decontaminated (100%), and 10 out of 10 health facilities were decontaminated (100%); no households, social economy enterprises, or schools received kits. Of 94 EDS alerts (including 10 postponements), 83 EDS were carried out (88.3%) and 68 bodies were swabbed.
- **North Kivu:** 12 out of 25 rings were opened (48.0%), 14 out of 40 households were decontaminated (35.0%), and 19 out of 25 ESSs (76.0%) were decontaminated; none of the 14 decontaminated households received kits, and 21 IPC scorecard assessments yielded an average score of 33%. Of 44 new EDS alerts and 44 reports, 45 EDSs were carried out and 44 bodies were swabbed.
- **Tshopo :** One ring was opened in Makiso-Kisangani; the PPE treatment center was decontaminated after a suspected case was seen; 11 replacement mattresses were distributed around confirmed cases, and 9 out of 9 health centers were monitored in 4 health zones, with 74 service providers briefed (100%). Cumulatively, 37 out of 37 health centers (100%) and 29 out of 33 households (87.9%) that had hosted a confirmed case were decontaminated within 48 hours.
- **Bas-Uélé :** 10 out of 10 ESS (100%) were monitored in Viadana (3), Ganga (5), Buta (1) and Monga (1), 25 out of 25 providers (100%) and 16 hygienists were briefed and 4 out of 5 triages (80.0%) are functional, the CS Rubi Lisanga (ZS of Buta) remaining non-functional; 8 out of 10 DLM are functional at the HGR of Buta and 5 out of 5 at the PoC Airport Zega, a preventive decontamination of the isolation room and the meeting room of the HGR of Buta was carried out and 6 beds and mattresses were given to the isolation room of Limba (WHO).

2.4.2. Challenges

In North Kivu, only 12 out of 25 infection control rings (48.0%) have been opened, with a significant gap in Katwa, Beni, and Mabalako. Household decontamination remains low (35.0%; 14/40) in Katwa, Beni, and Kalunguta, and the average infection prevention and control (IPC) score of assessed health facilities is capped at 33%. **In Tshopo**, 5 bodies could not be swabbed, IPC supplies and trained providers remain insufficient, and the incinerator at the Makiso-Kisangani General Referral Hospital is still out of service. **In Bas-Uélé**, the reporting of community death alerts remains low and delayed, and health facilities lack waste management areas, hygienic latrines, garbage cans, and handwashing stations.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri**, 75 new admissions were recorded compared to 58 discharges, including 4 recoveries. A total of 372 patients remained in isolation at the end of the day. The bed occupancy rate was **36.7%** ; 313 out of 353 patients (88.7%) received hot meals, and children under LNPE/SLM were fully covered at the CTE (11/11), in daycare (5/5), and in the community (41/41); 33 patients are in critical condition.
- **In North Kivu**, 61 new admissions were recorded, compared to 48 discharges (none recovered). A total of 382 patients remained hospitalized, including 263 in standardized facilities. The bed occupancy rate was **63.0%** ; 243 patients received hot meals, and 15 patients were in critical condition, all at the Beni Treatment Center.
- **In Tshopo**, 6 new admissions were recorded compared to 3. 20 patients remained in isolation at the end of the day, i.e. an occupancy rate of 64.5%.
- **In Bas-Uélé**, 6 new admissions were recorded and 52 were in isolation at the end of the daytime

- **In Haut-Uélé**, 14 new admissions were recorded. In total, 70 patients are in isolation in the CTEs; compared to 15 discharges, including 1 recovery, representing an occupancy rate of **50.7 %**.

2.5.2. Challenges

- **In Ituri**, the low capacity of the CTE/CT in Mambasa, Kilo, Mangala, Mandima and Lolwa and the lack of food support for 40 patients in Lolwa (19), Mambasa (14) and Mandima (7) remain a concern.
- **In Bas-Uélé**, the lack of a standardized CTE and ambulance for the transfer of patients remains a major operational challenge.
- **In Tshopo**, the lack of an ultrasound machine at the CTE, the insufficient number of beds at the treatment center
The experimental situation in Makiso-Kisangani and the demotivation of 12 out of 29 ambulance workers are weakening the care provided.

2.6. Vaccination

2.6.1. Main actions

- **In Bas-Uélé**, the vaccination of PPL and TPL continued in the health zones of Buta, Aketi and Viadana, with the pre-registration of religious leaders, RECO, traditional healers, motorcyclists and hunters, bringing the provincial total to 1,280 people vaccinated out of a target of 13,202 (9.7%), mainly in Buta (620; 21.6%) and in Ganga (273; 20.5%).
- **In Ituri**, the continuation of the Ervebo/Bundibugyo cross-immunity study and the vaccination of people living with HIV (PLHIV) remains contingent on the national Expanded Program on Immunization (EPI) receiving Ervebo vaccine. **In Tshopo**, 600 doses of Ervebo and 600 syringes have been received, and an additional 9,400 doses are expected this week.

2.6.2. Challenges

In Ituri, the target for PPL/TPL remains to be redefined, which prevents the calculation of a reliable coverage rate.

In Bas-Uélé, the UCCs have not been put into operation, the validated micro-plan and vaccination funds are not available.

2.7. Continuity of Care

2.7.1. Main actions • In Tshopo,

monitoring of the Boyoma Health Center (Makiso-Kisangani Health Zone) shows that at least 14 tuberculosis patients collect their medication there every day and that the incinerator is functional, but only 6 out of 38 staff (15.8%) are vaccinated, the laboratory technician is not trained in malaria diagnosis and the triage area is not effective; monitoring of PPL at the CTM and CTE as well as that of cured patients with comorbidities at the Makiso-Kisangani General Referral Hospital continued.

- **In Bas-Uélé**, the assessment of drug availability in the Ganga-Dingila health zone (May to August 2026) reveals a persistent shortage of Zinc + ORS, an increase in RHZE stocks (16,128 in August), and many ARV references remaining at zero.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri**, 23,782 home visits reached 105,316 people; 998 educational talks reached 20,218 people, 112 mass awareness campaigns reached 6,153 people, and 5 advocacy sessions reached 29 authorities and leaders. A total of 389 community alerts, including 142 deaths, were reported; 308 out of 319 feedback reports (96.6%) were collected, and 5 out of 6 incidents (83.3%) were resolved within 48 hours (11 out of 28 health zones completed, or 39.3%); the CREC facilitated the pre-listing of 311 contacts around 16 confirmed cases, the decontamination of 15 out of 15 households and 4 out of 4 facilities, 21 out of 21 EDS (Emergency Health Services) assessments, and the transfer of 35 out of 36 suspected cases (97.2%).
- **In North Kivu**, 10,654 home visits reached 34,721 people, 230 educational talks reached 7,449 people, and 63 mass communication sessions reached 8,368 people; 201 out of 750 expected community alerts (26.8%) were notified, 13 out of 17 feedback requests (76.5%) were resolved within 48 hours, and 910 out of 4,750 expected participants (19.2%) engaged in community actions. In Manguredjipa, a discussion was held with 22 members of pressure groups and resistance fighters, and a dialogue took place with 33 FARDC soldiers.

- **In Tshopo**, 47,270 people (21,092 men and 26,178 women) were sensitized during 7 mass sessions in Bafwasende, Tshopo, Mangobo and Makiso-Kisangani, 3 educational talks reached 187 people and a community debate, with the testimony of a healed person, brought together 104 people at the Christ-Roi parish in Mangobo; 4 broadcasts were held, 258 posters and 121 leaflets were distributed, the 3 identified refusals were lifted and the CREC supported the investigation in Makiso-Kisangani and the disinfection of households in Makiso, Mangobo and Tshopo.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In North Kivu**, 39 confirmed cases under follow-up, 223 new suspected cases, and 676 suspected cases under follow-up benefited from SMSPS interventions in the 10 health zones that reported (100%). The teams accompanied 31 out of 31 households with separated and orphaned children (100%), followed up with 5 out of 5 children in daycare and 26 out of 29 children in the community (89.7%), reintegrated 9 recovered individuals, supported 87 people living with HIV, provided psychoeducation to 409 people (117 men, 146 women, and 146 children), and announced 145 results, including 39 positive ones.
- **In Ituri**, 23 confirmed cases out of 57 (40.4%) from the previous day, 126 confirmed cases under observation out of 140 (90.0%), 23 new suspected cases out of 73 (31.5%), and 117 suspected cases under observation out of 128 (91.4%) benefited from SMSPS interventions. The teams supported 332 caregivers and 73 people living with HIV, monitored 13 out of 17 recoveries (76.5%) in the community and 15 out of 15 children in daycare, provided support to 6 out of 9 separated households, reintegrated 2 out of 4 recoveries, and announced 41 results, 23 of which were positive.
- **In Tshopo**, the 6 confirmed cases followed at the CTE Météo, including 1 new one, as well as 7 new suspects out of 7 and 4 suspects under follow-up out of 4 have been taken care of (100%); the teams supported 21 PPLs and 3 households, reintegrated 2 discharged people in the Kabondo health zone, carried out psychoeducation for 46 people, lifted 3 out of 3 resistances and announced 2 results, including 1 positive one; 5 out of 7 health zones that notified are covered (71.4%).
- **In Haut-Uélé**, only the CTEs of Isiro and Wamba reported: 2 new confirmed cases, 8 confirmed cases under follow-up and 1 suspected case under follow-up benefited from SMSPS interventions; the teams supported 13 caregivers, 31 PPLs and 13 children separated in the community, and announced 16 results, including 3 positive ones.
- **In Bas-Uélé**, SMSPS activities began with the arrival of a psychologist in the ZS of Viadana (Limba) and 1 result has been announced (negative).

2.9.2. Challenges

- **In Ituriseules**, 14 out of 28 affected health zones are covered (50.0%) and the Mandima health zone has no SMSPS coverage, which requires strengthening the capacities of psychologists and APS/TPS to ensure interventions around cases.
- **In Bas-Uélé**, SMSPS coverage remains embryonic, with activities having only just begun in the Viadana ZS.
- **In Tshopo**, there is a need to train the APS of the ZS of Wanie-Rukula and Bafwasende and a shortage of communication units and motorcycles for the movement of the pillar.
- **In Tshopo**, there is a need to train the APS (Auxiliary Security Personnel) of the ZS (Health Zones) of Wanie-Rukula and Bafwasende, and insufficient means of communication and movement of the pillar.

2.10. Logistics

2.10.1. Main actions

- **In Bas-Uélé**, provision of 6 beds and 6 mattresses to the CSR Limba for the isolation center and mattresses for the recovered (WHO), deployment of surveillance teams to Ganga and Viadana for contact tracing, alert verification and active search, and continuation of the rehabilitation of the meeting room and offices of the DPS as well as the construction of the transit center of the HGR of Buta, whose perimeter walls are completed.
- **In Tshopo**, monitoring of the construction of the Bafwasende CTE, the isolation of Konga-Konga and the extension of the Weather CTE, support for the mobility of the PCI, PEC, Communication and PoE/PoC pillars and allocation of a vehicle to the Weather CTE.

- In North Kivu, 9 out of 11 ambulances (81.8%) were mobilized and all 16 requested trips were completed all were successful (100%), for a response rate of 11 out of 28.

2.11. Security

2.12. Main actions

- In North Kivu, unidentified armed men attempted to set fire to the huts under construction at the Maboya PoC, and the threat remains persistent against the service providers of all the PoCs along the Butembo axis.

- In Ituri, health control of travelers at the PoC of Zunguluka (ZS of Mambasa) has been interrupted since October 3 at 3 p.m. following an attack attributed to ADF/NALU elements.

2.12.1. Main actions

2.13. PSEA (Prevention of Sexual Exploitation and Abuse)

2.13.1. Main actions

- In Ituri, 42 community, traditional and religious leaders (38 men and 4 women) were trained on the PSEAH in the Lita Health Zone and signed the code of conduct; 487 people (343 men, 86 women, 31 boys and 27 girls) were sensitized in Mahagi and the posting of messages continued in Bunia, Aru, Mongbwalu, Mambasa and Fataki, but only 13 out of 36 Health Zones (36.1%) know the reporting mechanism.

2.13.2. Challenges

At the national level, only 1 out of 7 DPS (14.3%) reported its PSEA activities; in Ituri, activities do not yet cover all affected health zones, mass awareness materials are lacking and the mapping of stakeholders remains incomplete.

KEY MESSAGES

- Active and sustained community transmission remains observed in the provinces of Ituri, North Kivu, and Haut-Uélé. Meanwhile, the province of Sud-Ubangi has not reported any confirmed cases in the last 24 hours, and a lull has been observed in the province of Tshopo compared to the last 72 hours.
- Progress to be consolidated has been made in the area of epidemiological investigations: All suspected cases were investigated (100%) within 24 hours of notification (**100 %**)
- Suspected cases were also investigated during the same period following their notification. However, the contact tracing rate is slightly higher than the previous day (**79.7% vs. 73.8%**). Nevertheless, efforts must be made to bring this rate up to the required threshold (95%).
- Strengthen surveillance, care, IPC, logistics and community engagement, prioritizing the increase in the capacities of CTE/CT in North Kivu, the rapid transfer of suspects and the isolation of confirmed cases.
- Strengthen preparedness and response to Ebola Virus Disease (EVD) in the river corridor and surrounding areas, particularly in South Ubangi, through active case finding, contact tracing, and ensuring the availability of supplies. Continue cross-border surveillance with South Sudan.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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